

ADMISSION NOTE - Patient synth_01 - Dengue fever with thrombocytopenia

CARE MULTISPECIALITY HOSPITAL, JAIPUR

ADMISSION NOTE

PATIENT NAME: Ankit Sharma, 28/M

IP NO: 24081203

ADMISSION DATE: 12/08/24

DIAGNOSIS:

Acute Febrile Illness with Thrombocytopenia - ?Dengue Fever. Rule out Enteric Fever.

CHIEF COMPLAINTS:

1. High-grade fever x 4 days
2. Severe body ache and headache x 4 days
3. Vomiting x 2 episodes

HISTORY OF PRESENT ILLNESS:

Patient, a 28-year-old male, was apparently well 4 days prior, when he developed high-grade fever, recorded up to 103°F, not associated with rigors but with chills. Fever is intermittent. This is associated with severe retro-orbital headache, generalized myalgia, and profound weakness. He also gives a history of 2 episodes of non-bilious vomiting one day prior. He has been taking TAB DOLO-650 on an SOS basis with partial and temporary relief. No history of rash, bleeding manifestations, cough, cold, or loose stools. Bowel and bladder habits are normal.

PAST HISTORY:

No known co-morbidities (DM/HTN/BA/TB). No known drug allergies. No prior surgeries or hospitalizations.

PERSONAL HISTORY:

Occupation: Software Engineer. Diet: Mixed. Appetite: Decreased. Sleep: Disturbed. Non-smoker, occasional alcohol use.

GENERAL EXAMINATION:

Patient is conscious, alert, and oriented to time, place, and person.

VITALS: Temp: 102.4°F, PR: 104/min, BP: 110/70 mmHg, RR: 18/min, SpO2: 99% on Room Air.

Pallor present. No icterus, cyanosis, clubbing, lymphadenopathy, or pedal edema.

SYSTEMIC EXAMINATION:

CVS: S1, S2 heard. No murmurs.

P/A: Abdomen soft, non-tender. Bowel sounds heard. No organomegaly.

RS: Bilateral air entry equal. No added sounds.

CNS: GCS 15/15. No focal neurological deficits. Neck soft.

INVESTIGATIONS:

Blood samples sent for: CBC, LFT, KFT, Urine R/M, Dengue NS1 Antigen, Dengue Serology (IgM/IgG), Widal Test. Reports awaited.

PLAN OF MANAGEMENT:

- Admit to ward.
- IV Fluids - Ringer's Lactate @ 125 ml/hr.
- INJ PARACETAMOL 1gm IV STAT & SOS.
- INJ EMESET 4mg IV SOS for vomiting.
- TAB PAN-40 40mg 1-0-0.
- Monitor vitals, intake-output, and watch for warning signs.
- Await investigation reports.

PROGRESS NOTE ? Day 1 - Patient synth_01 - Dengue fever with thrombocytopenia

CARE MULTISPECIALITY HOSPITAL, JAIPUR

PROGRESS NOTE

DATE: 13/08/24

PATIENT NAME: Ankit Sharma, 28/M

VITALS: PR 96/min, BP 116/70 mmHg, RR 18/min, SpO2 98% RA, Temp 100.2°F.

S: Patient reports feeling slightly better. Headache and myalgia have reduced in intensity. No further episodes of vomiting. Tolerating oral fluids.

O: Patient is conscious, oriented. Afebrile for the last 6 hours. Intake-output is adequate. Examination is unremarkable. No signs of bleeding or rash.

INVESTIGATIONS:

Dengue NS1 Antigen: POSITIVE.

CBC (13/08): Hb 14.2 g/dL, TLC 3,800/cumm, Platelets 85,000/cumm.

Platelet count has dropped from 1.1 lac/cumm on admission.

ASSESSMENT: Dengue Fever with mild thrombocytopenia. Currently in febrile phase.

CURRENT MEDICATIONS:

- IV Fluids - Ringer's Lactate @ 100ml/hr
- TAB DOLO-650 1-1-1
- TAB PAN-40 1-0-0

PLAN:

1. Continue supportive management with IV hydration.
2. Encourage good oral fluid intake (>3L/day).
3. Monitor vitals and watch for warning signs of severe dengue (persistent vomiting, abdominal pain, bleeding).
4. Repeat CBC in the morning to monitor platelet trend.

PROGRESS NOTE ? Day 2 - Patient synth_01 - Dengue fever with thrombocytopenia

CARE MULTISPECIALITY HOSPITAL, JAIPUR

PROGRESS NOTE

DATE: 14/08/24

PATIENT NAME: Ankit Sharma, 28/M

VITALS: PR 84/min, BP 120/80 mmHg, RR 16/min, SpO2 99% RA, Temp 98.8°F.

S: Patient feels much better today. No fever since yesterday afternoon. No headache or body aches. Reports good appetite.

O: Patient is afebrile, alert, and hemodynamically stable. Accepting diet well. No new complaints. Abdomen soft, no tenderness. No petechiae or rash.

INVESTIGATIONS:

CBC (14/08): Hb 14.5 g/dL, TLC 4,500/cumm, Platelets 65,000/cumm.

The platelet count has dropped further, which is expected (nadir phase), but the patient is clinically stable with no bleeding manifestations. Leucopenia is improving.

ASSESSMENT: Dengue Fever with thrombocytopenia. Entering critical/recovery phase. Clinically improving.

CURRENT MEDICATIONS:

- IV Fluids tapered and locked. To be removed.
- TAB DOLO-650 SOS
- TAB PAN-40 1-0-0

PLAN:

1. Discontinue IV fluids. Encourage oral hydration.
2. Monitor for 24 hours. If remains afebrile and stable with rising platelet trend tomorrow, can be planned for discharge.
3. Repeat CBC in the morning (15/08/24).

LABORATORY REPORT - Patient synth_01 - Dengue fever with thrombocytopenia

CARE MULTISPECIALITY HOSPITAL, JAIPUR - LABORATORY REPORT

PATIENT: Ankit Sharma, 28/M
IP NO: 24081203
REPORT DATE: 14/08/24

--- COMPLETE BLOOD COUNT ---
PARAMETER 12/08/24 13/08/24 14/08/24 REFERENCE RANGE
Hemoglobin (g/dL) 14.8 14.2 14.5 13.5 - 17.5
TLC (/cumm) 4,100 3,800 4,500 4,000 - 11,000
Platelet Count (/cumm) 110,000 85,000 65,000 150,000 - 450,000

--- SEROLOGY & IMMUNOLOGY ---
PARAMETER SAMPLE DATE RESULT REFERENCE
Dengue NS1 Antigen 12/08/24 POSITIVE Negative
Dengue IgM Antibody 12/08/24 PENDING Negative

--- MICROBIOLOGY ---
PARAMETER SAMPLE DATE STATUS
Widal Test 12/08/24 Report Awaited

MEDICATION RECORD - Patient synth_01 - Dengue fever with thrombocytopenia

PATIENT: Ankit Sharma, 28/M

IP NO: 24081203

--- ADMISSION MEDICATIONS (as on 12/08/24) ---

1. IV RINGER'S LACTATE - 1 pint @ 125 ml/hr
2. INJ EMESET 4mg - IV, SOS for vomiting
3. TAB DOLO-650 - 650mg, 1-1-1
4. TAB PAN-40 - 40mg, 1-0-0

--- DISCHARGE MEDICATIONS (as on 15/08/24) ---

1. TAB DOLO-650 - 650mg, SOS for fever/pain, for 5 days
2. TAB PAN-40 - 40mg, 1-0-0 (Before Food), for 5 days
3. TAB BECOSULES - 1 capsule, 1-0-0 (After Food), for 10 days

DISCHARGE ADVICE - Patient synth_01 - Dengue fever with thrombocytopenia

CARE MULTISPECIALITY HOSPITAL, JAIPUR
DISCHARGE SUMMARY

IP NO: 24081203
PATIENT NAME: Ankit Sharma, 28/M
DATE OF ADMISSION: 12/08/24
DATE OF DISCHARGE: 15/08/24

DIAGNOSIS:
1. Dengue Fever with Thrombocytopenia
2. Enteric Fever

COURSE IN THE HOSPITAL:
Mr. Ankit Sharma was admitted with complaints of high-grade fever, headache and body ache. Investigations revealed a positive Dengue NS1 antigen test and thrombocytopenia. He was managed conservatively with IV fluids for hydration and antipyretics. His fever subsided and he remained hemodynamically stable throughout the hospital stay. Platelet count showed an initial fall to a nadir of 65,000/cumm, and has shown a rising trend to 80,000/cumm on the day of discharge. He has been afebrile for more than 48 hours and is tolerating oral diet well. He is being discharged in a stable condition.

CONDITION AT DISCHARGE:
Afebrile, hemodynamically stable, accepting oral diet. Vitals: PR 80/min, BP 124/82 mmHg.

ADVICE ON DISCHARGE:
No.	Medication Name	Dosage	Frequency	Duration
1	TAB DOLO-650	650 mg	SOS (as needed for fever/pain)	5 Days
2	TAB PAN-40	40 mg	1 - 0 - 0 (Before Breakfast)	5 Days
3	TAB BECOSULES	1 Capsule	1 - 0 - 0 (After Breakfast)	10 Days

FOLLOW-UP INSTRUCTIONS:
- Take adequate rest for one week.
- Drink plenty of fluids (water, juice, ORS), at least 3-4 litres per day.
- Avoid NSAIDs (painkillers like Brufen, Voveran).
- Watch for warning signs (any bleeding, severe abdominal pain, persistent vomiting, black stools) and report to the hospital immediately if they occur.
- Follow-up with Dr. R. Gupta in the Medicine OPD after 3 days with a repeat CBC report.

PENDING RESULTS:
- Dengue IgM Antibody
- Widal Test